

SHOULDER SPECIAL TESTS

I. INSTABILITY TESTS

1. APPREHENSION TEST

Purpose

To assess anterior shoulder instability.

Procedure

Abduct the shoulder to 90°, flex the elbow to 90°, and slowly externally rotate the shoulder.

Positive Sign

Patient becomes apprehensive or fears shoulder dislocation.

Indicates

Anterior glenohumeral instability.

2. JERK TEST

Purpose

To assess posterior shoulder instability and posterior labral lesions.

Procedure

Shoulder abducted to 90° and internally rotated. Apply axial compression through the elbow while horizontally adducting the arm.

Positive Sign

Painful clunk or jerk.

Indicates

Posterior instability or posterior labral tear.

3. DRAWER TEST

Purpose

To assess anterior and posterior glenohumeral instability.

Procedure

Stabilize the scapula and translate the humeral head anteriorly and posteriorly.

Positive Sign

Excessive humeral head movement compared to the opposite side.

Indicates

Shoulder instability.

4. SULCUS SIGN

Purpose

To assess inferior glenohumeral instability.

Procedure

Apply downward traction to the relaxed arm.

Positive Sign

Visible sulcus below the acromion.

Indicates

Inferior or multidirectional shoulder instability.

II. IMPINGEMENT TESTS

1. HAWKINS-KENNEDY TEST

Purpose

Assess subacromial impingement.

Procedure

Flex shoulder and elbow to 90° and internally rotate the shoulder.

Positive Sign

Pain.

Indicates

Subacromial impingement.

2. NEER'S IMPINGEMENT TEST

Purpose

Assess subacromial impingement.

Procedure

Stabilize the scapula and passively flex the internally rotated arm.

Positive Sign

Pain.

Indicates

Subacromial impingement.

3. PAINFUL ARC TEST

Purpose

Assess shoulder impingement.

Procedure

Ask the patient to actively abduct the shoulder.

Positive Sign

Pain between 60°–120° of abduction.

Indicates

Subacromial impingement.

4. EMPTY CAN (JOB'S) TEST

Purpose

Assess supraspinatus tendon.

Procedure

Shoulder abducted to 90° in the scapular plane, internally rotated (thumb down), resist downward pressure.

Positive Sign

Pain or weakness.

Indicates

Supraspinatus tendinopathy or tear.

5. DROP ARM TEST

Purpose

Assess rotator cuff tear.

Procedure

Passively abduct the arm and ask the patient to slowly lower it.

Positive Sign

Arm drops suddenly or cannot be lowered smoothly.

Indicates

Supraspinatus tear/Rotator cuff tear.

III. LABRAL LESION TESTS

1. ACTIVE COMPRESSION TEST (O'BRIEN TEST)

Purpose

Assess SLAP lesion and AC joint pathology.

Procedure

Shoulder flexed to 90°, adducted 10–15°. Thumb pointing downward. Apply downward resistance. Repeat with palm facing upward.

Positive Sign

Pain or clicking that decreases with the palm-up position.

Indicates

SLAP lesion or AC joint pathology.

2. KIM TEST

Purpose

Assess posteroinferior labral lesion.

Procedure

Shoulder abducted to 90°. Apply axial compression with upward and posterior force while elevating the arm diagonally.

Positive Sign

Posterior shoulder pain.

Indicates

Posteroinferior labral tear.

IV. SCAPULAR DYSKINESIA TESTS

1. SCAPULAR DYSKINESIS TEST

Purpose

Assess abnormal scapular movement.

Procedure

Patient performs repeated shoulder flexion and abduction with light weights while the examiner observes scapular motion.

Positive Sign

Scapular winging or asymmetrical movement.

Indicates

Scapular dyskinesia.

2. SCAPULAR LOAD TEST (SCAPULAR ASSISTANCE TEST)

Purpose

Assess the effect of scapular assistance on shoulder movement.

Procedure

Examiner manually assists upward rotation and posterior tilt of the scapula during arm elevation.

Positive Sign

Pain decreases or shoulder movement improves.

Indicates

Scapular dyskinesia.

V. MUSCLE PATHOLOGY TESTS

1. YERGASON'S TEST

Purpose

Assess biceps tendon pathology.

Procedure

Patient supinates the forearm against resistance with the elbow flexed to 90°.

Positive Sign

Pain or snapping over the bicipital groove.

Indicates

Biceps tendinitis or transverse humeral ligament injury.

2. SPEED'S TEST

Purpose

Assess long head of biceps tendon.

Procedure

Shoulder flexed to 90°, elbow extended, forearm supinated; resist downward pressure.

Positive Sign

Pain in the bicipital groove.

Indicates

Biceps tendinitis or SLAP lesion.

3. LIFT-OFF TEST

Purpose

Assess Subscapularis muscle.

Procedure

Place the dorsum of the hand on the lower back and ask the patient to lift the hand away.

Positive Sign

Unable to lift the hand or marked weakness.

Indicates

Subscapularis tear or weakness.

4. TEST FOR SUPRASPINATUS

Purpose

Assess supraspinatus muscle.

Procedure

Perform the Empty Can Test against resistance.

Positive Sign

Pain or weakness.

Indicates

Supraspinatus pathology.

5. TEST FOR INFRASPINATUS

Purpose

Assess infraspinatus muscle.

Procedure

Patient externally rotates the shoulder against resistance with the elbow flexed to 90°.

Positive Sign

Pain or weakness.

Indicates

Infraspinatus tear or weakness.

6. HORNBLOWER'S SIGN

Purpose

Assess Teres Minor function.

Procedure

Shoulder elevated to 90° in the scapular plane with elbow flexed to 90°. Ask the patient to maintain external rotation.

Positive Sign

Unable to maintain the position or external rotation.

Indicates

Teres minor tear or severe rotator cuff pathology.

★ REVISION TABLE – SHOULDER SPECIAL TESTS

Category	Test	Indicates
Instability	Apprehension	Anterior instability
	Jerk	Posterior instability/Labral tear
	Drawer	Glenohumeral instability
	Sulcus Sign	Inferior instability
Impingement	Hawkins-Kennedy	Subacromial impingement
	Neer's	Subacromial impingement
	Painful Arc	Subacromial impingement
	Empty Can	Supraspinatus tear
	Drop Arm	Rotator cuff tear
Labral Lesions	O'Brien	SLAP lesion/AC pathology
	KIM	Posteroinferior labral tear
Scapular Dyskinesia	Scapular Dyskinesis Test	Scapular dyskinesia
	Scapular Load (Assistance) Test	Scapular dyskinesia
Muscle Pathology	Yergason's	Biceps tendinitis
	Speed's	Biceps tendinitis/SLAP
	Lift-Off	Subscapularis tear
	Supraspinatus Test	Supraspinatus pathology
	Infraspinatus Test	Infraspinatus pathology
	Hornblower's Sign	Teres Minor tear